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No. ———

Supreme Court, U.S.
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IN THE

Supreme Court of the United States

OCTOBER TERM, 1995

BARBARA A. DE BUONO, M.D., in her capacity as New York State Commisisoner of Health; KAREN SCHIMKE, in her capacity as New York State Executive Deputy Commissioner of Health; DEPUTY DIRECTOR, Division of Health Care Financing, Office of Health Systems Management, New York State Department of Health,

Petitioners,

against

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND, by its Trustees, JOHN BOWERS, JAMES CAPO, FRANK LONARDO, WILLIAM P. LYNCH, M. BRIAN MAHER, JAMES P. MELIA, GERALD OWENS, and PETER VICKERS,

Respondents.

Petition for a Writ of Certiorari to the United States Court of Appeals for the Second Circuit

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PETITION FOR A WRIT OF CERTIORARI

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QUESTION PRESENTED

Whether the Second Circuit's decision that section 514(a) of the Employee Retirement Income Security Act ("ERISA"), 29 U.S.C. § 1144(a) preempts N.Y. Pub. Health Law § 2807-d, which imposes a tax of six-tenths of one percent on the gross receipts from patient care services and other operating income of all hospials in New York State, insofar as it is applied to the income of medical centers owned and operated by an ERISA funds, is in conflict with, *inter alia, New York State Conference of Blue Cross & Blue Shield Plans v. Travelers Insurance Co.,* 115 S. Ct. 1671 (1995)?

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PETITION FOR A WRIT OF CERTIORARI

Petitioners Barbara A. De Buono, M.D.,[1] Commissioner of Health of the State of New York, Karen

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[1] Barbara A. De Buono, M.D., successor to David Axelrod, M.D. and then Mark Chassin, M.D., as Commissioner of Health of the State of New York and Karen Schimke, successor to Lorna H. McBarnette, Executive Deputy Commissioner of Health, have been automatically substituted as parties pursuant to Fed. R. App. P. 43. The position of Deputy Director, Division of Health Care Financing, currently is vacant.

Schimke, Executive Deputy Commissioner of Health of the State of New York, and the Deputy Director, Division of Health Care Financing, Office of Health Systems Management (collectively referred to herein as the "State"), respectfully pray that a writ of certiorari issue to review the judgment of the United States Court of Appeals for the Second Circuit, entered on January 4, 1996.[2]

OPINIONS BELOW

The opinion of the United States Court of Appeals for the Second Circuit is reported at 74 F.3d 28 (2d Cir. 1996) and reprinted in the appendix to the petition at 1a to 4a.[3] The order of the Supreme Court granting a writ of certiorari, vacating the judgment and remanding the case to the Second Circuit is reported at 115 S. Ct. 1819 (1995) and reprinted at 5a. The original opinion of the Second Circuit is reported at 27 F.3d 823 (2d Cir. 1994) and is reprinted at 6a to 14a. The memorandum opinion and order of the United States District Court for the Southern District of New York (Martin, J.) is not reported and is reprinted at 15a to 23a.

JURISDICTION

The judgment of the court of appeals was entered on January 4, 1996. This petition is filed within ninety days of the court's opinion. The jurisdiction of this Court is invoked pursuant to 28 U.S.C. § 1254(1) (1994).

STATUTES INVOLVED

The applicable statutes are reproduced in the appendix at 27a to 28a. The relevant federal statutory provision is section 514(a) of the Employee Retirement Income Security Act of 1974 ("ERISA"), 29 U.S.C. § 1144(a). The relevant New York State statutes are N.Y. Pub. Health Law §§ 2801(1), 2807-d(1)(a), and 2807-d (2)(c).

[2] The caption of this case in this Court contains the names of all parties to the proceeding.

[3] References to the appendix attached to the petition are cited as "—a." References to the joint appendix filed in the Second Circuit are cited as "JA ——."

STATEMENT OF THE CASE

A. The Hospital Assessment

In 1990, in order to offset increased health care costs, the New York State Legislature enacted section 2807-d of the New York Public Health Law, referred to by the courts below as the Health Facility Assessment or "HFA." 1990 N.Y. Laws, ch. 938.[4] The assessment was enacted as an alternative to substantial Medicaid payment reductions which would otherwise have been required (JA 51, 59).

The assessment is charged to all "hospitals" which are defined to encompass generally any facility engaged in providing services under the supervision of a physician or dentist, and includes diagnostic centers and treatment centers. N.Y. Pub. Health Law § 2801(1) (McKinney 1993). The assessment, which is six-tenths of one percent for diagnostic and treatment centers, applies with limited exceptions to a covered facility's gross receipts from patient care services and other operating income. N.Y. Pub. Health Law §§ 2807-d(1)(a), 2807-d(2)(c) (McKinney 1993). Assessable income has been further defined by the New York State Department of Health to include all cash receipts from patient care services irrespective of payment source, investment income, and other operating income, such as non-patient food sales, parking revenue, medical records fees, and the like (JA 65). Reports of assessable income and the assessments are required to be forwarded on a monthly basis to an administrative agent appointed by the New York State Depart-

[4] The assessment, as originally enacted, was to expire on April 1, 1992, but was amended to remain in effect after that date. 1995 N.Y. Laws, ch. 2, § 135.

ment of Health which, in turn, transmits those monies to the state's general fund (JA 57).

Respondents, trustees of the NYSA-ILA Medical and Clinical Services Fund (collectively referred to herein as the "Fund"), an ERISA employee welfare benefit plan, operate two medical centers in New York State. The medical centers are incorporated separately as New York not-for-profit corporations and are licensed by the New York State Department of Health as "diagnostic and treatment centers" pursuant to N.Y. Public Health Law § 2805 (JA 142-43). The centers provide medical, dental and related health care benefits on an outpatient basis to Fund participants and beneficiaries (JA 78-79). The medical centers also provide services to certain individuals who are not Fund participants, by virtue of an agreement between the medical centers and another employee benefit plan.

The medical centers' income is derived from a number of different sources, including employer contributions pursuant to collective bargaining agreements which are remitted to the Fund and transfered to the medical centers, employer contributions remitted directly to the medical centers, fee-for-service payments made directly to the medical centers by another ERISA fund, employee co-payments for services, fee-for-service payments from workers' compensation carriers, and miscellaneous sources such as supplier rebates and medical records fees (JA 80-82, 85).

The medical centers filed reports and paid assessments for the period from January 1, 1991 through November 30, 1991 (JA 84-85, 135-41). The centers did not include in their reported assessable income any monies which were transferred to them from the Fund reasoning that such income was akin to a grant which is non-assessable income under state law (JA 84). The State did not contest that exclusion. The centers reported as assessable income employer contributions to the centers,

contributions from workers' compensation carriers, fee-for-service payments from another ERISA fund, and employee payments for certain services, as well as miscellaneous income (JA 84). For the period from January 1, 1991 through November 30, 1991, the centers reported assessable income of $1,177,670 and paid assessments totaling $7,066.02 (JA 84-85). Thereafter, the centers discontinued filing reports and paid no further assessments (JA 86).

B. Prior Proceedings

On April 16, 1992, the Fund commenced this action for declaratory and injunctive relief and for a refund of the assessments already paid on the ground that the hospital assessment is preempted by ERISA (JA 18-23). The parties cross-moved for summary judgment pursuant to Fed. R. Civ. P. 56.

The district court granted the State's motion, determining that the hospital assessment is not preempted by ERISA because it is a law of general application with only an incidental economic impact on ERISA plans (21a).[5] Pointing to the *de minimis* amount of the assessment, the court distinguished this case from *The Travelers Insurance Co. v. Cuomo*, 813 F. Supp. 996 (S.D.N.Y.), *aff'd*, 14 F.3d 708 (2d Cir. 1993), *rev'd sub nom. New York State Conference of Blue Cross & Blue Shield Plans v. Travelers Insurance Co.*, 115 S. Ct. 1671 (1995), in which three New York State surcharges imposed on hospital bills of varying amounts of 9%, 11%, and 13%, depending on the identity of the third-party payor, were invalidated as ERISA-preempted (22a).

On June 23, 1994, the Second Circuit reversed the district court. The court stated that because "the HFA

[5] The district court did not reach the other issues raised by the State, namely, that the Tax Injunction Act, 28 U.S.C. § 1341 (1994), barred the complaint and that the claim for a refund is barred by the Eleventh Amendment to the United States Constitution.

targets only the health care industry" which is, "by definition, the realm where ERISA welfare plans must operate [the assessment] was bound to affect them" (12a). Thus, the court distinguished it from other laws of general applicability. Because the Fund had chosen to provide medical benefits through its own medical centers, the HFA operated as an immediate tax on payments and contributions which were intended to pay for medical benefits (12a).

Relying on its decision in *The Travelers Insurance Co. v. Cuomo*, 14 F.3d 708 (2d Cir. 1993), *rev'd sub nom. New York State Conference of Blue Cross & Blue Shield Plans v. Travelers Insurance Co.*, 115 S. Ct. 1671 (1995), the Second Circuit held that the assessment "relates to" the Fund directly by reducing assets otherwise available to provide health care benefits, thereby causing the Fund to reduce benefits provided or charge beneficiaries more (12a). According to the court, the impact of the assessment "will detrimentally affect the central mission and purpose of the Fund and hence cannot be characterized as remote and tenuous" (12a). Addressing the Fund's statement that it could avoid the tax altogether by funnelling all medical center income through the Fund, the court held that "such a restructuring would alter the Fund's established method of administraton" and would in itself be a basis for ERISA preemption (13a).

The State filed a petition for a writ of certiorari on October 21, 1994, after this Court granted the State's request for an extension of time within which to file the petition (26a). On May 1, 1995, this Court granted the petition, vacated the judgment, and remanded the case for further consideration in light of its decision in *New York State Conference of Blue Cross & Blue Shield Plans v. Travelers Insurance Co.*, 115 S. Ct. 1671 (1995) ("*Travelers*") (5a).

On January 4, 1996, the Second Circuit issued a decision which adhered to its original determination that the

HFA was preempted by ERISA insofar as it applies to the income of a medical center owned by an ERISA fund. The Second Circuit held that the facts of the case distinguish it from this Court's decision in *Travelers* because "there is no third party between the plan and the tax imposed pursuant to the HFA" (3a). Therefore, the statute "depletes the Fund's assets directly" (3a-4a). The court concluded its analysis by observing that "[n]othing in *Travelers* undermines our reliance on the HFA's direct economic impact upon the Fund as a basis for finding ERISA pre-emption nor precludes a determination that a state law's direct economic impact on ERISA plans is sufficient for a finding of pre-emption" (4a).

REASONS FOR GRANTING THE WRIT

I. THE DECISION OF THE COURT OF APPEALS IS IN DIRECT CONFLICT WITH SUPREME COURT PRECEDENT

This Court vacated the prior judgment in this case and remanded it to the court of appeals for reconsideration in light of its opinion in *Travelers*. The Second Circuit, however, did not actually apply the *Travelers* analysis in the decision. Instead, the court simply distinguished this case from *Travelers* on the basis that "there is no third party between the plan and the tax" (3a). An actual application of the *Travelers* analysis in this case demonstrates that the Second Circuit's opinion on remand is in conflict with this Court's decision in *Travelers*. Moreover, the court's reliance upon the lack of a third party between the generally applicable hospital assessment and the Fund as the basis for preemption is in conflict with *Mackey v. Lanier Collection Agency & Service, Inc.*, 486 U.S. 825 (1988). In *Mackey*, this Court held that a generally applicable garnishment statute was not preempted despite its direct imposition of legal obligations upon an ERISA plan which resulted in economic costs and administrative burdens to the plan.

In *Travelers*, this Court held that state laws which have only an indirect economic impact upon the choices made by ERISA plans with regard to the provision of benefits are not preempted by ERISA. Construing ERISA's preemption provision in light of its objective, "to avoid a multiplicity of regulation, in order to permit the nationally uniform administration of employee benefit plans," 115 S. Ct. at 1677-78, this Court held that state laws relate to plans and are preempted when they "mandate[] employee benefit structures or their administration," or provide "alternative enforcement mechanisms." 115 S. Ct. at 1678.

The Court found, however, that the challenged surcharges, which made Blue Cross a more attractive insurance alternative, only had "an indirect economic effect on choices made by insurance buyers, including ERISA plans." 115 S. Ct. at 1679. Such economic influence "does not bind plan administrators to any particular choice and thus function as a regulation of the ERISA plan itself." 115 S. Ct. at 1679. Thus, ERISA does not preempt state laws that have an impact "on the costs of benefits." 115 S. Ct. at 1679. This Court also noted that "nothing in the language of the Act or the context of its passage indicates that Congress chose to displace general health care regulation, which historically has been a matter of local concern." 115 S. Ct. at 1680.

The Second Circuit's original decision that the HFA is preempted rested upon three bases: first, that the HFA was unlike other laws of general applicability because it was health care regulation and therefore affected plans in "the realm where ERISA plans must operate," (12a); second, that the HFA directly depleted plan assets intended to provide health care benefits, (12a); and third, that a restructuring of the plan to avoid the tax would interfere with uniform administration of the plan

on an interstate basis, (13a). The first and third of these bases have since been explicitly rejected by this Court in *Travelers*. This Court found no congressional intent to have ERISA generally displace health care regulation, 115 S. Ct. at 1680, and also held that state laws do not relate to plans if they do not mandate plan decisions, even if the law's economic effect results in plans if they do not mandate plan decisions, even if the law's economic effect results in plans making different choices in different states. 115 S. Ct. at 1679.

The application of the *Travelers* analysis does not yield a different result in this case simply because the tax is applied to an ERISA plan-owned medical center and, therefore, the impact is characterized as "direct." In *Travelers*, this Court focused upon whether the surcharges' economic impact upon plans mandated benefits or structures, not upon whether the economic impact was directly or indirectly imposed upon plans. Like the surcharges in *Travelers*, the HFA does not bind plan administrators to a particular choice of how to provide benefits nor does it have anything other than an economic impact on plans.[6] The HFA arguably has an economic impact on a plan's decision to provide medical services in-kind, through a plan-owned medical center, rather than simply paying for medical services that plan participants have received elsewhere. However, in *Travelers*, this Court held that the surcharges were not preempted because they simply bear on the cost of benefits and the relative costs of competing insurance to provide them. The HFA, as it is applied to ERISA plan-owned medical centers, simply bears on the cost of benefits and the relative costs of competing *methods* to provide them.

Nothing in *Travelers* supports the conclusion that valid laws, such as the surcharges in *Travelers*, which indirectly

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[6] In fact, the tax has no economic impact on plan choices. Since the tax is imposed on all hospitals in New York, plans which do not own medical centers pay the tax as it is passed on as part of their participant's medical bills.

increase costs to plans through their application to third parties, become invalid when the plans happen to own or operate the third party. Indeed, the HFA is very similar to the 9% assessment in *Travelers* which is imposed upon the inpatient hospital costs paid by health maintenance organizations (HMOs). Under the Second Circuit's rationale, the 9% would be preempted if an HMO was owned by an ERISA plan. However, none of the reasons articulated by this Court for finding the 9% not preempted are in any way diminished simply because a plan chooses to own an HMO, rather than using the more usual method of purchasing its services.

The Second Circuit's decision rests solely on its characterization of the HFA as having a direct impact upon the Fund. This Court, however, has not construed ERISA as preempting laws simply because they directly operate upon plans. Even prior to *Travelers*, this Court, in *Mackey v. Lanier Collection Agency & Service, Inc.*, 486 U.S. 825 (1988), upheld a Georgia garnishment statute that directly operated upon plans. Like the HFA, the garnishment law was one of general application, but when it applied to plans it directly regulated their administrative procedures and, as a result, imposed costs. As garnishees, plan trustees were required to become parties to state lawsuits. They were also subject to state substantive law concerning spendthrift trusts. 486 U.S. at 831. Yet this Court held that these administrative burdens and costs, which were the result of a direct application of the statute to plans, were not enough to trigger preemption. 486 U.S. at 831. Moreover, in *Mackey*, this Court noted that run-of-the-mill state lawsuits against ERISA plans for unpaid rent, etc. were not preempted by ERISA. 486 U.S. at 833. Like the HFA, those legal obligations are applied directly to ERISA plans as well. The Second Circuit's failure to apply the *Travelers* analysis, and its reliance on a factor which this Court rejected as a basis for preemption in *Mackey*, has resulted in a decision in conflict with these cases. The Court should grant certiorari to resolve those conflicts.

II. THE DECISION OF THE COURT OF APPEALS IS ALSO IN CONFLICT WITH A DECISION OF THE EIGHTH CIRCUIT.

The decision of the Second Circuit is also in direct conflict with the decision of the Eighth Circuit in *Boyle v. Anderson*, 68 F.3d 1093 (8th Cir. 1995), *cert. denied sub nom. Boyle v. Smith*, 64 U.S.L.W. 3511 (March 18, 1996). In *Boyle*, the court found that a Minnesota provider tax on the gross patient revenues of hospitals is not preempted by ERISA. The statute contemplates that the tax will be paid by third party purchasers. It includes a pass-through provision that authorizes providers to transfer the expense as an addition to the amount due the provider pursuant to any provider contract. 68 F.3d at 1098 (citing Minn. Stat. § 295.582 (Supp. 1993)). The law also imposes direct obligations upon plans to pay the tax. It provides: "all third party purchasers of health care services . . . must pay the transferred expense." 68 F.3d at 1098 (citing Minn. Stat. § 295.582 (Supp. 1993)). Accordingly, no third party stands between ERISA plans and the legal obligation to pay the tax. Despite this, the court rejected a self-insured ERISA plan's challenge to the law.

In *Boyle*, the Eighth Circuit correctly examined the nature of the impact of the pass-through provisions of the law, rather than focusing on the direct or indirect source of the impact. First, the court correctly viewed the law as one of general application because the statute does not refer to ERISA plans, single them out for different treatment or apply to them differently than non-ERISA plans. 68 F.3d at 1101. Recognizing that the purpose of ERISA preemption is to permit nationally uniform plan administration, the Eighth Circuit examined the impact of the law on plan administration. The court held that the law only causes plans to pay more for healthcare services already covered. 68 F.3d at 1103. While it may add to the differences in the price of health care services from state to state, it will not require plans to be redesigned or

cause inefficiencies. 68 F.3d at 1105. With respect to the economic impact of the law, the Eighth Circuit cited this Court's *Travelers* decision extensively and noted that the impact of the surcharges in that case was much greater than Minnesota's 2% provider tax. 68 F.3d at 1108. Weighing these factors, the Eighth Circuit concluded that the law has only a tenuous, remote and peripheral impact on plans which did not warrant preemption.

The decision in *Boyle* cannot be reconciled with the Second Circuit's decision in this case. Both laws involve a generally applicable provider tax where, in certain situations, the legal obligation to pay the tax falls direcly on plans. In *Boyle*, the law had a direct impact on ERISA plans by requiring self-funded plans to pay the tax expense when transferred. The HFA has a direct impact on ERISA plans by requiring them to pay the tax when they own a medical center. In both cases there is a tax on payments which are the plan participants' medical benefits. Thus, both laws directly affect funds in their role as a provider of benefits. After application of an analysis consistent with *Travelers*, the Eighth Circuit correctly found the tax was not preempted. This Court should grant a writ of certiorari to resolve this conflict.

III. THIS CASE PRESENTS SUBSTANTIAL AND PRESSING QUESTIONS OF NATIONAL SIGNIFICANCE

This case is of great importance because it deals with the right of states to devise appropriate means to finance health care. In addition, the breadth of the Second Circuit's decision threatens all generally applicable laws which may apply to ERISA plan transactions in the health care field and beyond.

Providing comprehensive yet cost effective health care is an issue of national concern. Many states are exploring ways to address the problems of health care access,

equity in financing and coverage, and expenditure growth. One of the most pressing issues facing states seeking to ensure adequate health care coverage is how to pay for it and who will bear the financing burden. One method of financing the cost of health care is an assessment on hospitals or health care providers of the sort which New York State has enacted in N.Y. Pub. Health Law § 2807-d. At least fourteen other states utilize similar kinds of financing mechanisms.[7] Under the Second Circuit's reasoning, those states may not collect the tax from ERISA plan-owned medical facilities. They are not un-

[7] See Ala. Code §§ 40-26B-1, 40-26B-21 (1995) (imposing "providers of medical services privilege tax" on nursing facilities); Conn. Gen. Stat. § 12-263(b) (1994) (tax of 11% on "hospital gross earnings of each hospital"); Fla. Stat. Ann. § 395.7015 (West 1995) (imposing 1.5% assessment on annual net operating revenues of certain "health care entities"); Haw. Rev. Stat. § 346E-2 (1995) (taxes of 6% and 4% imposed on "nursing facility income" and "hospital income" respectively); Ill. Ann. Stat. ch. § 305 para. 5/5A-2 (Smith-Hurd 1993 & Supp. 1995) (assessment of 1.88% of "adjusted gross hospital revenue" of "each hospital provider"); Ky. Rev. Stat. Ann. § 142.303 (Michie 1995) (2.5% tax on gross revenues of providers "for the provision of hospital services"); Me. Rev. Stat. Ann. tit. 22, § 391 (West 1992 & Supp. 1995) (assessment of .15% of hospital's "gross patient service revenues"); Minn. Stat. Ann. § 295.52 (West 1991 & Supp. 1996) (2% tax on "gross revenues of each "hospital" and "health care provider"); Mont. Code Ann. § 15-60-102 (1995) ($2.80 "utilization fee" paid by each "nursing facility" for "each bed day in the facility"); N.H. Rev. Stat. Ann. § 84-A:2 (1991 & Supp. 1995) (tax "at a rate to be established by the legislature each biennium upon the gross patient services revenue of every hospital"); S.C. Code Ann. § 12-23-810 (Law. Co-op. Supp. 1995) (hospital tax based on total expenditures of each hospital as a percentage of total hospital expenditures state-wide); Utah Code Ann. § 26-36-105 (1995) (Medicaid Hospital Provider Temporary Assessment imposed on hospitals and surgical facilities); Vt. Stat. Ann. tit. 33, § 1953 (1991 & Supp. 1955) (4.02% "hospital assessment" imposed on each hospital's "audited gross inpatient revenues"); W.Va. Code Ann. §§ 11-27-9, 11-27-15 (1995) (health care provider tax of 2.5% of gross receipts on providers of "inpatient hospital services" and "outpatient hospital services").

common now and if the Second Circuit's decision stands, the number of such facilities surely will increase [8].

Preemption of provider taxes as they are applied to ERISA plan-owned providers could also threaten the validity of state regulatory schemes under federal law. As part of its 1991 amendments to the federal Medicaid Act, Congress explicitly authorized states to use taxes of general applicability, such as a sales tax extended to hospital charges, 42 U.S.C. § 1396a(t) (1994), and health care provider taxes, 42 U.S.C. § 1396b(w) (1994), as a means of raising revenue that qualifies for federal matching funds. However, an assessment must be broad based to qualify and an assessment is not broad based if it excludes any provider within a class. If a tax is not broad based, it must meet strict statistical parameters for a waiver or the entire tax is considered impermissible and the entire proceeds will be offset against a state's Medicaid expenditures resulting in a loss of federal matching funds.

The breadth of the Second Circuit's decision is sweeping. The assessment was invalidated because, the court stated, "there is no third party between the plan and the tax imposed pursuant to the HFA" (3a). Were this view to prevail there would be virtually no room for states to regulate transactions in which ERISA plans, among others, engage. While this case concerns a tax on a medical center owned by a plan, the court's analysis would logically require preemption of every state law regulating hospitals and other providers, to the extent they are imposed on ERISA plan-owned medical providers. For example, no third party stands between an ERISA plan-owned medical center and state imposed staffing requirements, rate regulation, medical and nursing standards, or sanitary condition regulations.

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[8] In New York City alone at least four other major union-affiliated ERISA funds operate diagnostic and treatment facilities similar to the ones involved in this case.

The Second Circuit's reasoning is not limited to ERISA plan-owned medical centers. ERISA plans own and operate day care centers, prepaid legal services plans and educational facilities. Thus, staffing requirements, safety requirements and sanitary requirements for plan operated day care centers might well be preempted. Interest on Lawyers Accounts ("IOLA") requirements imposed upon plan owned and operated prepaid legal services programs might well be preempted. Health and safety requirements imposed by the construction unions' plan-run training schools are also in jeopardy.

In addition, there is no third party between a plan and any generally applicable tax which is imposed upon ERISA plan transactions. Under this analysis real estate taxes paid by plans on plan-owned buildings, sales tax on items purchased by plans, day care licensing fees imposed upon plan-owned day care centers, unemployment taxes paid on behalf of plan employees and a host of other "direct" economic regulation are preempted.

In 1985, the Second Circuit warned that too expansive an interpretation of ERISA's preemption provision would result in a "fully insulated legal world that excludes these plans from regulation of any purely local transaction." *Rebado v. Cuomo*, 749 F.2d 133, 138 (2d Cir.), *cert. denied*, 472 U.S. 1008 (1985). The adoption and use of the Second Circuit's "direct impact" test to determine the scope of ERISA preemption will create such an insulated world and goes far beyond what Congress intended in enacting the preemption provision. The Court should grant the petition to review this important question.

CONCLUSION

For the foregoing reasons the petition for a writ of certiorari should be granted.

Respectfully submitted,

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Dated: New York, New York
April 3, 1996

APPENDICES

APPENDIX A

UNITED STATES COURT OF APPEALS
FOR THE SECOND CIRCUIT

No. 1294—August Term 1993

On remand from Supreme Court
(Submitted November 15, 1995 Decided Jan. 4, 1996)

Docket No. 93-7221

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND, by its Trustees, JOHN BOWERS, JAMES CAPO, FRANK LONARDO, WILLIAM P. LYNCH, M. BRIAN MAHER, JAMES P. MELIA, GERALD OWENS, and PETER VICKERS,

Plaintiffs-Appellants,

— v. —

DAVID AXELROD, M.D., in his capacity as New York State Commissioner of Health; LORNA H. MCBARNETTE, in her capacity as New York State Executive Deputy Commissioner of Health; STEVEN C. ANDERMAN, in his capacity as Deputy Director, Division of Health Care Financing, Office of Health Systems Management, New York State Department of Health,

Defendants-Appellees.

Before: WALKER and JACOBS, *Circuit Judges*, and ZAMPANO, *District* Judge.*

PER CURIAM:

On June 23, 1994, we entered a judgment and filed an opinion, *NYSA-ILA Medical & Clinical Servs. Fund v. Axelrod*, 27 F.3d 823, which reversed a summary judgment entered February 23, 1993 by the United States District Court for the Southern District of New York (John S. Martin, Jr., *Judge*). The district court had held that the Employee Retirement Income Security Act ("ERISA"), 29 U.S.C. § 1001 *et seq.*, did not preempt New York's Health Facility Assessment ("HFA"), N.Y. Pub. Health Law § 2807-d, reasoning that the tax imposed by the HFA "is not great enough to pose a serious economic threat to the plan." In reversing, we held that the HFA directly affected and was therefore "related to" the Fund in its principal role as an employee welfare plan.

On May 1, 1995, the Supreme Court entered an order granting certiorari, vacating the judgment of this court, and remanding the case to us for further consideration in light of *New York State Conference of Blue Cross & Blue Shield Plans v. Travelers Ins. Co.*, 115 S. Ct. 1671 (1995), *rev'g* 14 F.3d 708 (2d Cir. 1994). *See Chassin v. NYSA-ILA Medical & Clinical Servs. Fund*, 115 S. Ct. 1819 (1995).

We ordered counsel for the parties to brief the issue of whether *Travelers* requires a different result in this case. Upon receipt of these briefs, we carefully reconsidered this case in light of *Travelers*. For the reasons that follow, we adhere to our original decision reversing the judgment of the district court.

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* Honorable Robert C. Zampano, United States District Judge for the District of Connecticut, sitting by designation. Judge Zampano retired shortly after oral argument was heard in this case. Accordingly, the appeal was decided by the remaining panel members pursuant to Section 0.14 of the Local Rules of the Second Circuit.

The undisputed facts in this case have been set forth both in the district court opinion and in our original opinion, and we therefore do not fully restate them here. The Trustees of NYSA-ILA Medical & Clinical Services Fund (the "Fund") brought this suit seeking a declaration that ERISA preempts New York Public Health Law § 2807-d to the extent that the HFA taxes contributions and other payments for health care benefits received by two medical centers operated by the Fund. On appeal, we found that the HFA directly "relates to" the Fund in that "[t]he tax depletes those assets earmarked for the provision of health care benefits and, as a result, will cause the Fund to reduce benefits provided and/or to charge beneficiaries more in the future for benefits received." *Axelrod*, 27 F.3d at 827.

Upon reconsideration in light of the Supreme Court's holding in *Travelers*, we conclude that the ground upon which our earlier decision in this case rested remains undisturbed. *Travelers* involved a challenge to New York's imposition of a surcharge on hospital bills paid by commercial insurers and Health Maintenance Organizations but not on hospital bills paid by Blue Cross/Blue Shield insurers. The statute at issue in *Travelers* did not impose surcharges on ERISA plans themselves, but on insurance carriers providing health insurance coverage to the plans and their beneficiaries. The surcharge consequently raised costs to the patient, who remained liable for the balance of any bill an ERISA plan or insurance carrier refuses to pay. The Supreme Court concluded that "a law operating as an indirect source of merely economic influence on administrative decisions [by ERISA plans], as here, should not suffice to trigger preemption." *Travelers*, 115 S. Ct. at 1680.

The facts of the present case distinguish it from *Travelers*, because, unlike in *Travelers*, there is no third party between the plan and the tax imposed pursuant to the HFA. Rather, the statute at issue depletes the Fund's

assets directly, and thus has an immediate impact on the operations of an ERISA plan. As we observed in our original opinion, the HFA imposes "an immediate tax on payments and contributions which were intended to pay for participants' medical benefits . . . [and] thus directly affects the Fund in its principal role as an employee welfare benefit plan." *Axelrod*, 27 F.3d at 827.

Nothing in *Travelers* undermines our reliance on the HFA's direct economic impact upon the Fund as a basis for finding ERISA pre-emption nor precludes a determination that a state law's direct economic impact on ERISA plans is sufficient for a finding of pre-emption. Because our reasoning is not inconsistent with the Supreme Court's opinion in *Travelers*, we adhere to our original disposition that reversed and remanded this case.

APPENDIX B

SUPREME COURT OF THE UNITED STATES

No. 94-745

MARK CHASSIN, *et al.*,
Petitioners,
v.
NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND, ETC., *et al.*,

May 1, 1995

On petition for writ of certiorari to the United States Court of Appeals for the Second Circuit. The petition for writ of certiorari is granted. The judgment is vacated and the case is remanded to the United States Court of Appeals for the Second Circuit for further consideration in light of *New York State Conference of Blue Cross & Blue Shield Plans v. Travelers Insurance Company*. 514 U.S. —, 115 S.Ct. 1671, —— L.Ed.2d —— (1995).

APPENDIX C

UNITED STATES COURT OF APPEALS FOR THE SECOND CIRCUIT

No. 1294—August Term 1993

(Argued March 10, 1994 Decided June 23, 1994)

Docket No. 93-7221

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND, By Its Trustees, JOHN BOWERS, JAMES CAPO, FRANK LONARDO, WILLIAM P. LYNCH, M. BRIAN MAHER, JAMES P. MELIA, GERALD OWENS, and PETER VICKERS,
Plaintiffs-Appellants,

—v.—

DAVID AXELROD, M.D., in his capacity as New York State Commissioner of Health; LORNA H. MCBARNETTE, in her capacity as New York State Executive Deputy Commissioner of Health; STEVEN C. ANDERMAN, in his capacity as Deputy Director, Division of Health Care Financing, Office of Health Systems Management, New York State Department of Health,
Defendants-Appellees.

Before:

WALKER and JACOBS, *Circuit Judges*,
and ZAMPANO, *District Judge*.*

---

* Honorable Robert C. Zampano, United States District Judge for the District of Connecticut, sitting by designation. Judge Zampano retired shortly after oral argument was heard in this case. Accordingly, the appeal was decided by the remaining panel members pursuant to Section 0.14 of the Local Rules of the Second Circuit.

WALKER, Circuit Judge:

The Trustees of NYSA-ILA Medical & Clinical Services Fund (the "Fund") brought this suit seeking a declaration that the Employee Retirement Income Security Act ("ERISA"), 29 U.S.C. § 1001 *et seq.*, preempts New York Public Health Law § 2807-d to the extent the latter provision taxes contributions and other payments for health care benefits received by two medical centers operated by the Fund. The Fund now appeals from a judgment of the United States District Court for the Southern District of New York (John S. Martin, Jr., *Judge*) holding that ERISA does not preempt the state tax and granting summary judgment to defendant state health care officials. We reverse.

BACKGROUND

The Fund is an ERISA-regulated multiemployer, labor-management trust fund established through collective bargaining to administer an employee welfare benefit plan that provides medical, dental, and other health care services to ERISA plan participants, consisting primarily of longshore workers, and their dependents. The Fund provides these health care benefits through three medical centers that it owns and operates, two in New York and one in New Jersey. The centers provide treatment exclusively to plan participants and beneficiaries.

In 1990, the State of New York enacted a Health Facility Assessment ("HFA"), which imposed a tax on the gross receipts from patient care services and general operations of all hospitals in New York State. N.Y. Pub. Health Law § 2807-d. The HFA was enacted to help eliminate the State's budget deficit for the 1990-91 fiscal year. The HFA became effective January 1, 1991, and was supposed to expire on April 1, 1992, but has subsequently been amended to remain in force on and after that date. 1992 N.Y. Laws, ch. 41, §§ 54, 163. The HFA applies to a wide range of medical facilities, includ-

ing diagnostic and treatment centers such as the medical centers operated by the Fund. N.Y. Pub. Health Law § 2801(1). The amount of the assessment, which varies by type of facility, is six-tenths of one percent for the Fund's medical centers. *Id.* § 2807-d(2)(c). For purposes of the assessment, "gross receipts" are defined as "all monies received for or on account of hospital and health related services" with only limited exceptions. *Id.* § 2807-d(3)(c).

The Fund's medical centers receive income from a variety of sources. The bulk of their costs are covered by employer contributions remitted to the Fund which are then transferred to the centers on an as-needed basis to pay for the centers' payroll and other operating expenses. However, the centers obtain additional revenue from certain employer payments made directly to the centers, employee co-payments, other fee-for-service payments, patient registration fees, payments from workers' compensation carriers, and such miscellaneous sources as the sale of scrap or waste, fees for reproducing medical records, and supplier rebates. For the period from January 1, 1991 to November 30, 1991, the Fund's two New York medical centers reported combined assessable income of $1,177,670 and paid assessments totalling $7,066. The Fund discontinued filing HFA reports with the New York State Department of Health after November 1991 and has paid no assessments since that time.

The medical centers did not include in their assessable income monies transferred from the Fund to the centers, and the State defendants have not contested the exclusion of these payments in this case. However, the question of whether monies transferred from the Fund to the centers are assessable has never, to our knowledge, been litigated or definitively resolved. Since early 1992, the Fund has restructured its payment procedures so that certain employer contributions and registration fees that were formerly paid directly to the medical centers are now remitted to the Fund itself. The Fund believes that this

restructuring avoids the effect of the HFA, but is unwilling to redirect to the Fund all payments now received by the medical centers until this case has been decided.

The Fund instituted this suit in April 1992 asserting that ERISA preempts the HFA insofar as it imposes a levy on contributions and payments for health care benefits provided by the Fund. The Fund does not challenge imposition of the HFA on the centers' miscellaneous revenue earned from activities unrelated to the health care services provided to participants and beneficiaries, such as the income derived from the sale of scrap, records reproduction fees, and supplier rebates. The Fund sought declaratory and injunctive relief as well as restitution of a substantial portion of the HFA assesments it had previously paid.

On cross-motions for summary judgment, the district court ruled that the HFA is a tax of general application that is not preempted by ERISA because it has only an incidental impact on ERISA plans. The district court reasoned that the minimal tax imposed by the HFA "is not great enough to pose a serious economic threat to the plan which might trigger preemption." *NYSA-ILA Medical & Clinical Servs. Fund v. Axelrod*, 92 Civ. 2279 (JSM), slip op. at 8 (S.D.N.Y. Feb. 18, 1992). Accordingly, the district court judgment dismissing the action and this appeal ensued. We review the district court's decision *de novo*. *See Westinghouse Elec. Corp. v. New York City Transit Auth.*, 14 F.3d 818, 821 (2d Cir. 1994).

DISCUSSION

ERISA was enacted in 1974 "to promote the interests of employees and their beneficiaries in employee benefit plans," including both pension and welfare plans. *Shaw v. Delta Air Lines, Inc.*, 463 U.S. 85, 90 (1983). An "employee welfare benefit plan" is defined in the statute as

> any plan, fund, or program . . . established or maintained by an employer or by an employee organization, or by both, to the extent that such plan, fund, or program was established or is maintained for the purpose of providing for its participants or their beneficiaries, through the purchase of insurance or otherwise, . . . medical, surgical, or hospital care or benefits, or benefits in the event of sickness, accident, disability, death or unemployment . . . .

29 U.S.C. § 1002(1). The parties do not dispute that the Fund qualifies as an employee welfare benefit plan that fulfills its institutional mission by providing medical benefits directly to plan participants and their beneficiaries through Fund-operated medical centers.

One of ERISA's main objectives was to eliminate state regulation of employee benefit plans. *See Ingersoll-Rand Co. v. McClendon*, 498 U.S. 133, 138 (1990); *Shaw*, 463 U.S. at 99 (quoting legislative history). To accomplish this, Congress enacted an express preemption clause that is "deliberately expansive." *Pilot Life Ins. Co. v. Dedeaux*, 481 U.S. 41, 46 (1987). It provides that ERISA "shall supersede any and all State laws insofar as they may now or hereafter relate to any employee benefit plan" covered by the law. 29 U.S.C. § 1144(a). The Supreme Court has observed that "[a] law 'relates to' an employee benefit plan, in the normal sense of the phrase, if it has a connection with or reference to such a plan." *Shaw*, 463 U.S. at 96-97. Preemption thus applies even where a state law was not specifically designed to regulate the terms and conditions of employee benefits plans or does not relate to those subjects covered by ERISA, such as reporting, disclosure, and fiduciary responsibilities. *See Ingersoll-Rand Co.*, 498 U.S. at 137-39; *FMC Corp. v. Holliday*, 498 U.S. 52, 58-59 (1990); *Shaw*, 463 U.S. at 98. Indeed, even where a state law has no express link to an employee benefit plan, it can be preempted " 'insofar as' the law applies to benefit plans in particular cases." *Morgan Guar. Trust Co. v. Tax Appeals Tribunal*, 80 N.Y.2d 44, 49 (1992) (quoting *Shaw*, 463 U.S. at 97 n.17).

ERISA preemption is not without limitations, however. The Supreme Court has recognized that ERISA preemption will not apply to state laws that "affect employee benefit plans in too tenuous, remote, or peripheral a manner to warrant a finding that the law 'relates to' the plan." *Shaw*, 463 U.S. at 100 n.21. When it announced this limitation, the Court "express[ed] no views about where it would be appropriate to draw the line," *id.*, but it has subsequently observed that this limitation will apply "with many laws of general applicability," *District of Columbia v. Greater Wash. Bd. of Trade*, 113 S. Ct. 580, 583 n.1 (1992). There are a number of cases that bear out this observation. *See, e.g., Mackey v. Lanier Collection Agency & Serv., Inc.*, 486 U.S. 825 (1988) (generally applicable garnishment law not preempted); *Fort Halifax Packing Co. v. Coyne*, 482 U.S. 1 (1987) (state law requiring one-time severance payment not preempted); *Aetna Life Ins. Co. v. Borges*, 869 F.2d 142 (2d Cir.) (state escheat law not preempted when applied to recover unclaimed ERISA plan benefits), *cert. denied*, 493 U.S. 811 (1989); *see also Matter of Dyke*, 943 F.2d 1435, 1447 (5th Cir. 1991) (collecting cases). Unfortunately, these decisions have provided no sure method for determining whether the HFA's impact on the Fund is too "tenuous, remote, or peripheral" to trigger ERISA preemption.

The State argues that the HFA is a state tax law of general application that affects ERISA plans only incidentially. According to the State, the HFA is no more directed at ERISA plans than laws implementing sales taxes, utility taxes, and gasoline taxes, and should thus be seen simply as a "cost of doing business" within the state. Relying on *Mackey* and *Aetna Life Ins. Co.*, the State contends that the HFA should not be preempted because it covers ERISA plans only by virtue of the plans' inclusion in a broader class of entities subject to regulation.

However, we believe that the HFA is distinguishable from these laws of general application. The HFA does not apply broadly to every sector of society in New York; nor does it apply to the Fund simply because it engaged in certain kinds of activities which any other member of society might engage in, such as purchasing gasoline or paying one's employees. Rather, the HFA targets only the health care industry. Because this industry is, by definition, the realm where ERISA welfare plans must operate, the HFA was bound to affect them. In a case such as the present one where the plan has chosen to provide medical benefits through self-run medical centers, the HFA operates as an immediate tax on payments and contributions which were intended to pay for participants' medical benefits. The HFA thus directly affects the Fund in its principal role as an employee welfare benefit plan. It does not touch the Fund only slightly on the outer limits of its plan activities; it affects the very operations and functions that make the Fund what it is, a provider of medical, surgical, and hospital care to its participants and their beneficiaries.

We thus conclude that the HFA "relates to" the Fund and does so in a very direct way. The tax depletes those assets earmarked for the provision of health care benefits and, as a result, will cause the Fund to reduce benefits provided and/or to charge beneficiaries more in the future for benefits received. Both impacts will detrimentally affect the central mission and purpose of the Fund and hence cannot be characterized as remote or tenuous. *See Travelers Ins. Co. v. Cuomo*, 14 F.3d 708, 720 (2d Cir. 1993) ("[T]he surcharges here force ERISA plans to increase either plan costs or reduce plan benefits. Therefore, they have the requisite connection to ERISA."), *petition for cert. filed*, 62 U.S.L.W. 3625 (U.S. Mar. 9, 1994) (No. 93-1414); *E-Systems, Inc. v. Pogue*, 929 F.2d 1100, 1103 (5th Cir.) ("The cost of the plan must therefore increase for the employer and/or employees or the benefits must be adjusted downwards to offset the tax bite.

This is the type of impact Congress intended to avoid when it enacted the ERISA legislation."), *cert. denied*, 112 S. Ct. 585 (1991).

Our decision would remain the same even if we assumed that the Fund could avoid the HFA by restructuring its finances so that funds formerly remitted by plan participants and employers directly to the medical centers would be remitted to the Fund which would subsequently transfer them to the centers. Wholly apart from whether the transmission of assets from the Fund to the centers could be taxed without running afoul of ERISA preemption, an issue we need not add and do not decide, such a restructuring would alter the Fund's established method of administration and would require it to implement procedures for its New York facilities that differ from those of its New Jersey facility. Moreover, these changes would not be insignificant since they would affect the Fund's primary means of collecting contributions and paying for benefits. A state law that "relates to" an ERISA plan is not saved from ERISA preemption by the fact that a plan could restructure its operations to avoid the impact of the state law. Requirements under state law that force plans to change their traditional methods of administration or to adopt different methods on a state-by-state basis are features of state regulation that ERISA was designed to prevent. *See FMC Corp.*, 498 U.S. at 59-60; *Alessi v. Raybestos-Manhattan, Inc.*, 451 U.S. 504, 524 (1981); *National Carriers' Conference Comm. v. Heffernan*, 454 F. Supp. 914, 918 (D. Conn. 1978) (Newman, J.), *Compare Aetna Life Ins. Co.*, 869 F.2d at 147 (discounting Fund's complaint that it would need to make administrative and accounting adjustments in order to comply with state escheat law because the impact was not substantial enough).

The district court acknowledged that the HFA will directly impact the Fund by leaving less money for benefits, but determined that the magnitude of the impact was too insubstantial to trigger preemption since the tax rate

was only .6%. We do no agree with this reasoning. The magnitude of a statute's economic impact on employee benefit plans may enter the analysis where the statute's connection to the ERISA plan is otherwise insufficient for preemption. For example, a real estate gains tax that has only a peripheral connection to ERISA plans may still be subject to preemption if the high rate of tax has a substantial economic impact on a plan. *See Morgan Guar. Trust Co.*, 80 N.Y.2d at 51-53 (ten percent tax on gain from ERISA plan's sale of real property preempted because of direct impact on plan's administration and investment strategy). However, the converse is not true. A statute that "relates to" ERISA plans cannot escape preemption simply because the magnitude of the impact is thought to be insubstantial. Adopting such an approach would contravene the policy that underlies the preemption provision of protecting ERISA plans from state regulation and would open the door to endless litigation about what constitutes a "substantial" impact.

Travelers Insurance Co. v. Cuomo, supra, is not to the contrary. In that case, we held that New York surcharges on hospital rates were preempted by ERISA to the extent ERISA plans must pay surcharges. In the course of our analysis, we discussed Judge Martin's opinion in this case without criticizing it. However, we cited it only for the proposition that "*a substantial* economic impact, standing alone, could be enough to bring ERISA's preemption clause into play." 14 F.3d at 721. We did not endorse—and now expressly refuse to endorse—the converse proposition urged by the State: that a regulation which has a *de minimis* economic impact on ERISA plans may withstand preemption regardless of what plan operations it affects.

For the foregoing reasons, we reverse the judgment of the district court.

APPENDIX D

UNITED STATES DISTRICT COURT
SOUTHERN DISTRICT OF NEW YORK

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92 Civ. 2779 (JSM)

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND BY ITS TRUSTEES: JOHN BOWERS, JAMES CAPO, FRANK LONARDO, WILLIAM P. LYNCH, M. BRIAN MAHER, JAMES P. MELIA, GERALD OWENS, AND PETER VICKERS,
Plaintiff,

-v.-

DAVID AXELROD, M.D., IN HIS CAPACITY AS NEW YORK STATE COMMISSIONER OF HEALTH; LORNA H. MCBARNETTE, IN HER CAPACITY AS NEW YORK STATE EXECUTIVE DEPUTY COMMISSIONER OF HEALTH; AND STEVEN C. ANDERMAN, IN HIS CAPACITY AS DEPUTY DIRECTOR, DIVISION OF HEALTH CARE FINANCING, OFFICE OF HEALTH SYSTEMS MANAGEMENT, NEW YORK STATE DEPARTMENT OF HEALTH,
Defendants.

MEMORANDUM OPINION AND ORDER

JOHN S. MARTIN, JR., District Judge:

Plaintiff and defendants have moved for summary judgment. The central question presented by these motions is whether the Employment Retirement Income Security Program (ERISA), 29 U.S.C.A. §§ 1001-1461 (1985 & Supp. 1992), preempts the State of New York's Health Facility Assessment (HFA), N.Y. PUB. HEALTH LAW § 2807-d (McKinney Supp. 1992).

The HFA, a state tax of general application, is an assessment charged on gross receipts of hospitals. *Id.* § 2807-d(1). Plaintiff is an ERISA plan which operates medical centers for the benefit of its plan beneficiaries. New York currently applies the HFA to plaintiff's medical centers. Plaintiff claims that its medical centers should not be subject to the tax because ERISA preempts state laws, such as the HFA, which affect ERISA plans. Plaintiff also seeks to recover the tax it has thus far paid under the HFA.

Defendant's move for dismissal on grounds that the Tax Injunction Act, 28 U.S.C.A. § 1341 (1976), prevents this Court from ordering injunctive relief which interferes with a state tax plan. Defendants also move in the alternative for summary judgment on plaintiff's claims.

Defendants' motion for summary judgment is granted because ERISA does not preempt the Health Facility Assessment.

DISCUSSION

Defendants first question this Court's jurisdiction over this case. ERISA provides that actions concerning ERISA preemption of state laws shall be brought in the United States District Courts. 29 U.S.C.A. § 1132(e)(1). However, this jurisdictional grant is in conflict with a jurisdictional bar contained in the Tax Injunction Act (TIA). 28 U.S.C.A. § 1341 (1976). The TIA provides that "[t]he district courts shall not enjoin, suspend or restrain the assessment, levy or collection of any tax under State law where a plain, speedy and efficient remedy may be had in the courts of such State." *Id.* The conflict is resolved, however, in light of ERISA's provision that "nothing in [this subchapter] shall be construed to alter, amend, modify, invalidate, impair, or supersede any law of the United States . . . or any rule or regulations issued under any such law." 29 U.S.C.A. § 1144(d). This provision prevents ERISA from superseding the TIA. *Retirement*

Fund Trust v. Franchise Tax Bd., 909 F.2d 1266 (9th Cir. 1990); *Ashton v. Cory*, 780 F.2d 816, 822 (9th Cir. 1986); *National Carriers' Conference Comm. v. Heffernan*, 440 F. Supp. 1280, 1283 (D. Conn. 1977); *In re Sterling Die Casting Co., Inc.*, 118 B.R. 205, 207 (Bkrtcy. E.D.N.Y. 1990); *see also Shaw v. Delta Air Lines, Inc.*, 463 U.S. 85, 104-05, 103 S. Ct. 2890, 2904 (1983).

Plaintiff raises additional arguments against the TIA's applicability to this action which are unpersuasive. The Fund urges this Court to use rules of statutory construction to find that ERISA supersedes the TIA. Plaintiff notes that a more specific statute overrides one that is more general. *Morgan v. Mancari*, 417 U.S. 535, 550-51 (1974). However, the *Morgan* rule only applies where there is no clear expression by Congress as to whether the later statute is intended to supersede the earlier one. *Id.* This rule is inapplicable in this case because of the clear language of ERISA § 1144 which indicates that Congress did not want ERISA to affect other Federal law.

Plaintiff also argues that it should be allowed the benefit of an exception to the TIA. The Supreme Court has ruled that the United States is not restricted by the TIA from protecting itself and its instrumentalities from unconstitutional state exactions in federal courts. *Department of Employment v. United States*, 385 U.S. 355, 358 (1966). The Court extended this rule to allow Indian tribes an exemption from the TIA if the United States could have sued on their behalf. *Moe v. Confederated Salish and Kootenai Tribes*, 425 U.S. 463, 96 S. Ct. 1634 (1976). *Moe* grants this exception to Indian tribes in part because of the unique trustee relationship of the United States to the tribes. *Id.* at 473, 96 S. Ct. at 1641; *Federal Deposit Ins. Corp. v. State of New York*, 928 F.2d 56, 60 (2d Cir. 1991). Plaintiff misconstrues *Moe* to state that any private party is exempt from the TIA's jurisdictional bar if the United States could have sued on its behalf. *Federal Deposit Ins. Corp.*, 928 F.2d at 60. Since plaintiff is not

in a trust relationship with the United States, the *Moe* exception to the TIA does not apply in this action.

There being no reason extrinsic to the TIA to bar its application to this action, it is now appropriate to consider the terms of the TIA itself. The TIA operates to bar federal courts from granting injunctions or declaratory judgments in matters affecting state tax schemes when there is a "plain, speedy and efficient remedy" to be had in state courts. *Keleher v. New England Tel. & Tel. Co.*, 947 F.2d 547, 549 (2d Cir. 1991).

Plaintiff's arguments to the contrary notwithstanding, the HFA does fall within the class of state taxes excluded from the district courts' jurisdiction. For TIA purposes, the Second Circuit broadly defines the word "tax." *Keleher*, 947 F.2d at 549. Any state or local revenue collection device is a "tax" for purposes of the TIA. *Id.* (citing Robinson Protective Alarm Co. v. City of Philadelphia, 581 F.2d 371, 374-76 (3d Cir. 1978)). In the case at bar, the HFA is a revenue collection device of the state and is therefore a tax for TIA purposes.

Once it has been established that a state tax if at issue, federal courts do not have jurisdiction to enjoin its enforcement or declare it invalid if there is a plain, speedy and efficient remedy available in the state courts. 28 U.S.C.A. § 1341. It is not enough that the defendant may raise the invalidity of the tax as a defense to an enforcement proceeding. *Georgia R.R. & Banking Co. v. Redwine*, 342 U.S. 299, 303 n.11, 72 S.Ct. 321, 324 n.11 (1952); *National Carriers' Conference Comm. v. Heffernan*, 440 F. Supp. 1280, 1283 (D. Conn. 1977).

In New York State Courts, there are two affirmative proceedings that the taxed entity might pursue; an Article 78 proceeding and an action for declaratory judgment. However, where constitutional questions are implicated in an Article 78 proceeding, the proper procedure is for the action to be converted to one for declaratory judgment

because an Article 78 proceeding is not an appropriate forum for constitutional challenges. *Board of Educ. v. Gootnick*, 427 N.Y.S.2d 777 (N.Y. 1980). Thus, in New York, an action for declaratory judgment is the only action available to the Fund to affirmatively challenge the constitutionality of the HFA.

Plaintiff asserts, however that its claim will be dismissed in state court because ERISA grants exclusive jurisdiction over ERISA claims to the federal courts. Yet, plaintiff's speculation on the fate of its action in the state courts is discredited in light of the fact that New York State courts regularly entertain such actions. *See, e.g., Matter of Morgan Guar. Trust Co. v. Tax Appeals Tribunal*, 587 N.Y.S.2d 252 (N.Y. 1992); *Tap Elec. Contracting Serv., Inc. v. Harnett*, 556 N.Y.S.2d 988 (N.Y. 1990); *Sasso v. Vachris*, 494 N.Y.S.2d 856 (N.Y. 1985); *United Health Servs., Inc. v. Upstate Admin. Servs., Inc.*, 573 N.Y.S.2d 851 (N.Y. Sup. Ct. 1991); *People v. Art Steel Co., Inc.*, 505 N.Y.S.2d 715 (N.Y. Crim. Ct 1986). Nevertheless, courts have held that the uncertainty over whether New York State courts will entertain ERISA preemption actions renders the availability of a state court remedy anything but "plain." *Travelers Ins. Co. v. Cuomo*, No. 92 Civ. 3999 (LJF) (S.D.N.Y. Feb. 3, 1993); *Heffernan*, 440 F. Supp. at 1283. There is no need to decide this issue, here however, because, reaching the merits of plaintiffs, claim, I conclude that the HFA is not preempted by ERISA.

Since there are no disputed issues of material fact, this case is an appropriate one to resolve on motion for summary judgment. This case is governed by recent Second Circuit pronouncements on the extent of ERISA preemption. Although the parties make much of analogizing to, and discrediting, the Second Circuit's holding in *Rebaldo v. Cuomo*, 749 F.2d 133 (2d Cir. 1984), *cert. denied*, 472 U.S. 1008, 105 S. Ct. 2702 (1985), that case does

not provide the clearest guidance. The Circuit Court has since refashioned its test for ERISA preemption in light of Supreme Court rulings subsequent to *Rebaldo*, although it has not abandoned *Rebaldo*. *See Rogers v. Consolidated Rail Corp.*, 948 F.2d 858 (1991); *Aetna Life Ins. Co. v. Borges*, 869 F.2d 142 (2d Cir. 1989), *cert. denied*, 493 U.S. 811, 110 S. Ct. 57 (1989).

While ERISA's preemption provision is to be interpreted broadly, the presumption against preemption of state law is at its strongest when dealing with a law that is within the states' traditional police powers. *Rogers*, 948 F.2d at 859; *Aetna*, 869 F.2d at 144. The Second Circuit, in conducting a survey of ERISA preemption cases has generalized from them to find that there are certain types of cases which fall on either side of the preemption line. *Aetna*, 869 F.2d at 145-46. The Circuit Court found

> that the laws that have been ruled preempted are those that provide an alternative cause of action to employees to collect benefits protected by ERISA, refer specifically to ERISA plans and apply solely to them, or interfere with the calculation of benefits owed to an employee. Those that have not been preempted are laws of general application—often traditional exercises of state power or regulatory authority—whose effect on ERISA plans is incidental.

Id. at 146. Further defining "what constitutes an incidental impact," the Circuit Court stated that indirect economic impact alone was not enough. The impact must affect the "primary administrative functions of benefit plans, such as determining an employee's eligibility for a benefit and the amount of that benefit." *Id.*

Using this litmus test, the Second Circuit held in *Aetna* that a Connecticut escheat law was not preempted by ERISA. *Id.* at 147. The escheat law required that pen-

sion benefits which remained unclaimed for more than twenty years be paid to the state. ERISA had no provision for what to do with the unclaimed benefits. The court recognized that the state escheat law would have an economic impact on the benefit plan and that administrative and accounting adjustments would have to be made to comply with the state law. However, the Circuit Court held that "these indirect and administrative effects are not substantial enough [ ] to persuade us that this is the type of law that Congress intended to preempt." Thus, "[a]bsent more specific direction from Congress, we can safely assume that Congress did not intend to create an exception to this traditional exercise of state authority." *Id.*

In the case at bar, this Court is confronted with the question of ERISA preemption of a state's tax law, a traditional exercise of state authority. The HFA is a tax of general application; it is not specifically directed at ERISA plans but applies to all health care facilities. The tax does not interfere with the calculation of benefits or the determination of an employee's eligibility for benefits. The impacts on benefit plans are incidentally economic resulting only in the need for administrative and accounting procedures to comply with the law.

The fact that the tax will leave less money for benefits is not decisive here as it was not in *Aetna*, where Connecticut's escheat laws also drained funds that might otherwise go to benefits. The tax is not great enough to pose a serious economic threat to the plan which might trigger preemption. *See Aetna, 869 F.2d at 147.* Here the tax is "sixth-tenths of one percent of each facility's gross receipts received from all patient care services and other operating income on a cash basis." N.Y. PUB. HEALTH LAW § 2807-d(2)(c). The Fund paid, for the period of January 1 through November 30, 1991, $7,066.02 as the assessment on $1,177,670 of income which is subject to the assessment out of approximately $20,000,000

(twenty million) total income for 1991. Fier Affidavit ¶ 19. While there are no numerical standards established by case law to determine how much tax is too much, it is clear that the HFA is not significant enough to warrant preemption of a state's traditional taxing power without explicit instructions from Congress.

Plaintiff notes that the New York Court of Appeals has reached a different conclusion. *Morgan*, 587 N.Y.S.2d 252. The Court of Appeals held that a state tax on gains from the transfer of real property was preempted by ERISA because it drained plan funds, caused an administrative burden that would be different than that in other states, and impacted the structure of administrative decisions on the disposal of assets. *Id.* at 256-57. However, *Morgan* is distinguishable from the case before this Court. The tax in *Morgan* was ten percent (10.0%) of the consideration received for the property sold, an amount the Court of Appeals considered significant, *Id.* at 257, as opposed to six-tenths of one percent (0.6%) in the case before this Court. The economic impact of the HFA is not as substantial as that found in *Morgan*. *Compare Travelers Ins. Co. v. Cuomo*, No. 92 Civ. 3999 (LJF) (S.D.N.Y. Feb. 3, 1993) (state surcharges of 9%, 11%, and 13% on hospital rates preempted by ERISA to the extent that ERISA plans must pay the surcharges).

Thus, it is consistent with decisions of the New York Court of Appeals, the Second Circuit, and District Courts within it, to hold that ERISA does not preempt the HFA.

Therefore, plaintiff's motion for a refund of the approximately $7,000 paid as HFA assessments is dismissed.

CONCLUSION

Defendants' motion for summary judgment dismissing the complaint is granted because ERISA does not preempt the Health Facility Assessment. Accordingly, the

remainder of defendants' and plaintiff's motions are dismissed as moot.

SO ORDERED.

Dated: February 18, 1993
New York, New York

/s/ John S. Martin, Jr.
JOHN S. MARTIN, JR.
U.S.D.J.

APPENDIX E

UNITED STATES DISTRICT COURT
SOUTHERN DISTRICT OF NEW YORK

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92 Civ. 2779 (JSM)

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND BY ITS TRUSTEES: JOHN BOWERS, JAMES CAPO, FRANK LONARDO, WILLIAM P. LYNCH, M. BRIAN MAHER, JAMES P. MELIA, GERALD OWENS, AND PETER VICKERS,
Plaintiff,

-against-

DAVID AXELROD, M.D., IN HIS CAPACITY AS NEW YORK STATE COMMISSIONER OF HEALTH; LORNA H. MCBARNETTE, IN HER CAPACITY AS NEW YORK STATE EXECUTIVE DEPUTY COMMISSIONER OF HEALTH; AND STEVEN C. ANDERMAN, IN HIS CAPACITY AS DEPUTY DIRECTOR, DIVISION OF HEALTH CARE FINANCING, OFFICE OF HEALTH SYSTEMS MANAGEMENT, NEW YORK STATE DEPARTMENT OF HEALTH,
Defendants.

JUDGMENT

Plaintiff and defendants having moved for summary judgment, and the said motions having come before the Honorable JOHN S. MARTIN, U.S.D.J., and the Court thereafter on February 23, 1993, having handed down its memorandum opinion and order (#71096); granting defendants' motion for summary judgment dismissing the complaint because ERISA does not preempt the Health Facility Assessment, and dismissing as moot the remainder of defendants' and plaintiff's motions, it is,

ORDERED, ADJUDGED AND DECREED: That defendants' motion for summary judgment dismissing the complaint be and it is hereby granted, and it is further,

ORDERED, that the complaint be and it is hereby dismissed because ERISA does not preempt the Health Facility Assessment, and it is further,

ORDERED, that the remainder of defendants' and plaintiff's motions be and they are hereby dismissed as moot.

DATED: New York, New York
March 1, 1993

/s/ James M. Parkison
Clerk

APPENDIX F

SUPREME COURT OF THE UNITED STATES

No. A-180

MARK CHASSIN, *et al.*,
Petitioner

v.

NYSA-ILA MEDICAL AND CLINICAL SERVICES FUND, *et al.*

ORDER

UPON CONSIDERATION of the Application of counsel for the petitioner,

IT IS ORDERED that the time for filing a petition for a writ of certiorari in the above-entitled case, be and the same is hereby, extended to and including October 21, 1994.

/s/ Clarence Thomas
Associate Justice of the Supreme
Court of the United States

Dated this 14th day of September, 1994

APPENDIX G

STATUTES

29 U.S.C. § 1144. Other Laws

(a) Supersedure; effective date

Except as provided in subsection (b) of this section, the provisions of this subchapter and subchapter III of this chapter shall supersede any and all State laws insofar as they may now or hereafter relate to any employee benefits plan described in section 1003(a) of this title and not exempt under section 1003(b) of this title. This section shall take effect on January 1, 1975.

* * * *

N.Y. Pub. Health Law § 2801(1)

* * * *

1. "Hospital" means a facility or institution engaged principally in providing services by or under the supervision of a physician or, in the case of a dental clinic or dental dispensary, of a dentist, for the prevention, diagnosis or treatment of human disease, pain, injury, deformity or physical condition, including, but not limited to, a general hospital, public health center, diagnostic center, treatment center, dental clinic, dental dispensary, rehabilitation center other than a facility used solely for vocational rehabilitation, nursing home, tuberculosis hospital, chronic disease hospital, maternity hospital, lying-in-asylum, out-patient department, out-patient lodge, dispensary and a laboratory or central service facility serving one or more such institutions, but the term hospital shall not include an institution, sanitarium or other facility engaged principally in providing services for the prevention, diagosis or treatment of mental disability and which is subject to the powers of visitation, examination, inspection and investigation of the department of mental hygiene except for those distinct parts of such a facility

which provide hospital service. The provisions of this article shall not apply to a facility or institution engaged principally in providing services by or under the supervision of the bona fide members and adherents of a recognized religious organization whose teachings include reliance on spiritual means through prayer alone for healing in the practice of the religion of such organization and where services are provided in accordance with those teachings.

* * * *

N. Y. Pub. Health Law § 2807-d. Hospital assessments

1. (a) Hospitals, as defined in this article, excluding hospitals specified in paragraph (b) of this subdivision, are charged assessments on their gross receipts from all patient care services and other operating income, less pesronal needs allowances and refunds, on a cash basis in the percentage amounts and for the periods specified in subdivision two of this section. Such assessments shall be submitted by or on behalf of hospitals to the commissioner or his designee.

* * * *

2. (c) For al other facilities issued an operating certificate pursuant to section twenty-eight hundred five of this article, including diagnostic and treatment centers, the assessments shall be six-tenths of one percent of each facility's gross receipts received from all patient care services and other operating income on a cash basis beginning January first, nineteen hundred ninety-one for hospital or health-related services, including diagostic and treatment center services.

* * * *